

Behavioral Health Levels of Care

CMT: CMT-0006 (SR)

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Introduction

Levels of behavioral health care differ in terms of the extent of physical security, hours of operation, and range of services provided. Although length of stay sometimes is included as an additional defining characteristic in descriptions of levels of care, that approach is not taken here because the peer-reviewed literature includes descriptions of both long-term and short-term applications of all levels of care. Also, although level of care might be represented by a continuum rather than discrete categories, discrete categories are discussed here because it is in terms of discrete definitions that the efficacious uses of various levels of care have been studied in clinical trials.

Levels of Care

- **Inpatient care** is the behavioral health level of care that offers the highest level of physical security and most intensive psychiatric and nursing intervention. Inpatient psychiatric units, whether they are located in general hospitals or freestanding behavioral health facilities, are generally locked and staffed by nurses around the clock. Designated involuntary units have specific facility design, procedures, and staff training to provide care in accordance with legal criteria and court review. A covering physician is available to assess patients if indicated around the clock.(1)(2)(3)(4) Specialty centers providing inpatient treatment for substance use disorders usually do so in a freestanding inpatient facility with medical management available.(5)(6)(7)(8) Withdrawal management may be provided in a freestanding detoxification center or by intensive medical management in an inpatient medical or psychiatric unit depending on need.(5)(6)(7)(8)
- **Residential care** is intended for patients who need an around-the-clock care setting but do not need the high level of physical security and frequency of psychiatric and nursing intervention available on an inpatient unit. Patients admitted to residential care usually are voluntary and unlikely to need physical restraint.(3)(4)(6)(7)(8)(9)(10)(11) The treatment team is generally composed of the same mix of professionals as on an inpatient unit, but the frequency and intensity of psychiatric, medical, and nursing interventions, and the number of hours a day of behavioral health counseling vary depending on program emphasis and patient needs. Specialty centers providing residential treatment may do so in a variety of ways, including programs that provide around-the-clock behavioral health care in therapeutic rehabilitation facilities and therapeutic communities; residential treatment centers, which can support active (but not around-the-clock) medical monitoring but can easily and quickly make medical or nursing care available if the need arises; and medically managed programs (often described as subacute care), which support patients who need around-the-clock medical or nursing monitoring but who are not anticipated to require the intensive treatment and resources of a licensed hospital.(6)(7)(8)(9) Withdrawal monitoring may be provided in a variety of 24-hour care environments, and the setting of care is dependent on multiple factors, including the presence of comorbidities (eg, medical, psychiatric, or behavioral comorbidities that may interfere with treatment and recovery), the degree of active medical or nursing monitoring required, whether or not medication is necessary to manage current withdrawal symptoms, and the likelihood of completing acute withdrawal management in a monitored setting and entering into chronic addiction treatment.(5)(6)(7)(8)(9)(12)
- **Observation care** spans the gap between outpatient and inpatient care.(13)(14)(15)(16) Observation care can be provided in a psychiatric emergency services unit, holding unit, or other facility-based setting, or in a dedicated observation care unit or other hospital-based setting.(17)(18) The purpose of observation care is to provide time for patient testing, monitoring, or treatment that will help determine whether inpatient care is warranted. In this way, observation care is to be used for patients who are "close calls" in terms of need for inpatient care. This is consistent with the definition of observation care provided by the Centers for Medicare and Medicaid Services (CMS). According to CMS, "Observation care is a well-defined set of specific, clinically appropriate services, which include ongoing short-term treatment, assessment, and reassessment, that are furnished while a decision is being made regarding whether patients will require further treatment as hospital inpatients or if they are able to be discharged from the hospital." The duration of observation care is not uniformly defined. Contractual agreements (eg, between payers and providers) or regulatory language (eg, CMS) often set the time frame of observation care.(19) Appropriate use of MCG content presumes that observation care duration is finite, and that failure of a patient to improve sufficiently within the given time indicates appropriateness for admission to a specific behavioral health care setting.
- **Crisis intervention care** consists of short-term crisis stabilization with a very short, time-limited approach (eg, 1 to 3 days) that can be conducted in a facility setting (eg, residential care, short-term psychiatric unit) or by outreach teams.(20)(21)(22)
- **Partial hospital programs (PHPs)** provide multidisciplinary behavioral care, typically for 6 to 8 hours per day, 5 to 7 days per week, and are staffed similarly to the day shift of an inpatient unit.(23) Like residential care, treatment in PHPs is generally conducted by

the same mix of behavioral health professionals as inpatient care.(4)(6)(7)(8)(9)(23) For voluntary patients with urgent behavioral health conditions who have sufficient community support and do not require around-the-clock behavioral care, PHPs represent a less restrictive alternative to inpatient care, and their efficacy has been supported in randomized controlled trials (RCTs).(24) For almost all the behavioral health diagnosis categories, the term PHP refers only to programs that are tasked primarily with the treatment of acute behavioral health conditions (as opposed to day treatment programs, which provide a mix of psychosocial treatment, education, and recreational activities to patients with nonurgent behavioral health conditions).(23)

- **Intensive outpatient programs (IOPs)** typically provide 3 to 4 hours of psychosocial treatment, 1 to 4 days per week (generally for 6 to 12 hours a week), mostly by using a group format, and are intended for circumstances when a patient needs a type or frequency of psychosocial treatment that is not available in a standard outpatient setting. An IOP may or may not provide pharmacotherapy or nursing care.(3)(4)(6)(7)(8)(9)(10)(23)(25)
- **Outpatient care** consists of pharmacologic and/or psychosocial treatment services provided in office or clinic settings (or through consultation to an assisted living or other special care program or facility) for managing the care of patients with behavioral health disorders. Patients at this level of care are generally seen at least twice a year, but visit frequencies are flexible and determined on the basis of individual patient needs.(4)(6)(7)(8)(9)(26)(27)
- **Day treatment care** consists of a mix of psychosocial treatment, education, and recreational activities for patients with nonurgent behavioral health conditions (as opposed to the multidisciplinary care provided to patients with acute behavioral health conditions in PHPs).(3) This level of care typically is monitored by staff whose licensure and credentials (eg, nurses, social workers) are allowed for this role by local statutes and are clinically appropriate to patient needs.(3)(28)(29)(30) Specialty centers providing outpatient treatment for substance use disorders commonly do so in day treatment centers.(5)
- **Long-term community-based residential care** consists of extended-care skilled services provided on a daily or near daily basis (at least 5 days a week) by or under the supervision of licensed healthcare professionals (eg, nurses, psychologists, social workers) for patients with chronic serious mental illness.(31)(32) Examples of settings in which this care may take place include, but are not limited to, group homes and recovery residences (eg, halfway house, sober living home). These services are sometimes funded by public or government programs and may not be covered by all payers.(4)(33)
- **Custodial care** consists primarily of support for non-health related services (eg, ADL, IADL), rather than provision of skilled services.(34)(35) Custodial care services may be delivered in a variety of settings, including long-term care facilities, assisted living facilities, group or care homes, private duty nursing or aide services, respite care, and adult day care.(36)(37)
- **Recovery facility care** consists of treatment in subacute (medical or rehabilitation/skilled nursing) facilities, and care consists of ongoing skilled medical treatments and moderate-level to low-level intensive therapy. The focus of the recovery facility may be on skilled nursing interventions, rehabilitation therapy, or a combination of both. A patient in a recovery facility generally requires less physician involvement than does a patient in an inpatient rehabilitation facility.(38)(39)
- **Home healthcare** designates services provided to patients who are homebound, often with medical comorbidities. Service is provided by an RN, with additional services provided by a social worker, licensed behavioral health professional, or other professional available through the home healthcare agency.(40)

References

1. Kivler CA. Staffing inpatient psychiatric units. *Journal of the American Psychiatric Nurses Association* 2012 Jan-Feb;18(1):30-1. DOI: 10.1177/1078390311432313. [Context Link 1] View abstract...
2. American Psychiatric Nurses Association. APNA position statement: staffing inpatient psychiatric units. *Journal of the American Psychiatric Nurses Association* 2012 Jan-Feb;18(1):16-22. DOI: 10.1177/1078390311432133. (Reaffirmed 2022 Jun) [Context Link 1] View abstract...
3. Kober D, Martin A. Inpatient psychiatric, partial hospital, and residential treatment for children and adolescents. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:3797-3803. [Context Link 1, 2, 3, 4, 5]
4. Level of care placement. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. *ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions*. 3rd ed. Carson City, NV: The Change Companies; 2013:174-306. [Context Link 1, 2, 3, 4, 5, 6]
5. Addressing withdrawal management and intoxication management. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. *ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions*. 3rd ed. Carson City, NV: The Change Companies; 2013:127-173. [Context Link 1, 2, 3, 4]
6. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08] [Context Link 1, 2, 3, 4, 5, 6, 7, 8]
7. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25] [Context Link 1, 2, 3, 4, 5, 6, 7, 8]
8. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct. [Context Link 1, 2, 3, 4, 5, 6, 7, 8]
9. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11] [Context Link 1, 2, 3, 4, 5, 6]
10. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep. [Context Link 1, 2]

11. Houston M, et al. Principles of Care for Treatment of Children and Adolescents with Mental Illnesses in Residential Treatment Centers. [Internet] American Academy of Child and Adolescent Psychiatry. 2010 Jun Accessed at: <https://www.aacap.org/>. [accessed 2023 Sep 26] [Context Link 1]
12. Reif S, et al. Residential treatment for individuals with substance use disorders: assessing the evidence. *Psychiatric Services* 2014;65(3):301-312. DOI: 10.1176/appi.ps.201300242. [Context Link 1] View abstract...
13. Baugh CW, Suri P, Caspers CG, Granovsky MA, Neal K, Ross MA. Financial viability of emergency department observation unit billing models. *Academic Emergency Medicine* 2019;26(1):31-40. DOI: 10.1111/acem.13452. [Context Link 1] View abstract...
14. Blecker S, Gavin NP, Park H, Ladapo JA, Katz SD. Observation Units as Substitutes for Hospitalization or Home Discharge. *Annals of Emergency Medicine* 2016;67(6):706-713.e2. DOI: 10.1016/j.annemergmed.2015.10.025. [Context Link 1] View abstract...
15. Southerland LT, et al. Beyond observation: protocols and capabilities of an emergency department observation unit. *American Journal of Emergency Medicine* 2019;37(10):1864-1870. DOI: 10.1016/j.ajem.2018.12.049. [Context Link 1] View abstract...
16. Mace SE. Care of special populations in an observation unit: Pediatrics and Geriatrics. *Emergency Medicine Clinics of North America* 2017;35(3):685-699. DOI: 10.1016/j.emc.2017.03.011. [Context Link 1] View abstract...
17. Ross MA, Graff LG. Principles of observation medicine. *Emergency Medicine Clinics of North America* 2001;19(1):1-17. [Context Link 1] View abstract...
18. Venkatesh AK, Geisler BP, Gibson Chambers JJ, Baugh CW, Bohan JS, Schuur JD. Use of observation care in US emergency departments, 2001 to 2008. *PLoS ONE* 2011;6(9):e24326. DOI: 10.1371/journal.pone.0024326. [Context Link 1] View abstract...
19. Centers for Medicare and Medicaid Services. Medicare Claims Processing Manual. Chapter 12 - Physicians/Nonphysician Practitioners Rev.11842 [Internet] Centers for Medicare and Medicaid Services. 2023 Feb 09 Accessed at: <https://www.cms.gov/manuals/>. [accessed 2023 Sep 11] [Context Link 1]
20. Hamm MP, et al. A systematic review of crisis interventions used in the emergency department: recommendations for pediatric care and research. *Pediatric Emergency Care* 2010;26(12):952-62. DOI: 10.1097/PEC.0b013e3181fe9211. [Context Link 1] View abstract...
21. Wolff A. Development of a psychiatric crisis stabilization unit. *Journal of Emergency Nursing* 2008;34(5):458-9. DOI: 10.1016/j.jen.2008.05.008. [Context Link 1] View abstract...
22. Murphy SM, Irving CB, Adams CE, Waqar M. Crisis intervention for people with severe mental illnesses. *Cochrane Database of Systematic Reviews* 2015, Issue 12. Art. No.: CD001087. DOI: 10.1002/14651858.CD001087.pub5. [Context Link 1] View abstract...
23. Rosser J, Michael S. Partial Hospitalization Programs and Intensive Outpatient Programs. 2021 AABH Standards and Guidelines [Internet] Association for Ambulatory Behavioral Healthcare. 2021 Accessed at: <https://aabh.org/>. [accessed 2023 Sep 12] [Context Link 1, 2, 3, 4]
24. Marshall M, Crowther R, Sledge WH, Rathbone J, Soares-Weiser K. Day hospital versus admission for acute psychiatric disorders. *Cochrane Database of Systematic Reviews* 2011, Issue 12. Art. No.: CD004026. DOI: 10.1002/14651858.CD004026.pub2. [Context Link 1] View abstract...
25. McCarty D, et al. Substance abuse intensive outpatient programs: assessing the evidence. *Psychiatric Services* 2014;65(6):718-726. DOI: 10.1176/appi.ps.201300249. [Context Link 1] View abstract...
26. Lam RW, et al. Canadian network for mood and anxiety treatments (CANMAT) 2016 clinical guidelines for the management of adults with major depressive disorder: section 1. disease burden and principles of care. *Canadian Journal of Psychiatry* 2016;61(9):510-23. DOI: 10.1177/0706743716659416. [Context Link 1] View abstract...
27. Vieta E, Valenti M. Pharmacological management of bipolar depression: acute treatment, maintenance, and prophylaxis. *CNS Drugs* 2013;27(7):515-29. DOI: 10.1007/s40263-013-0073-y. [Context Link 1] View abstract...
28. Garrison D, Daigler GE. Treatment settings for adolescent psychiatric conditions. *Adolescent Medicine Clinics* 2006;17(1):233-50. DOI: 10.1016/j.admecli.2005.10.002. [Context Link 1] View abstract...
29. Sussman S. A review of Alcoholics Anonymous/ Narcotics Anonymous programs for teens. *Evaluation & the Health Professions* 2010;33(1):26-55. DOI: 10.1177/0163278709356186. [Context Link 1] View abstract...
30. Vanderploeg JJ, Franks RP, Plant R, Cloud M, Tebes JK. Extended day treatment: a comprehensive model of after school behavioral health services for youth. *Child and Youth Care Forum* 2010;38(1):5-18. DOI: 10.1007/s10566-008-9062-6. [Context Link 1] View abstract...
31. Am I eligible for nursing facility care services? WAS 388-106-0355. Title-Chapter-Section 388-106-0355 [Internet] State of Washington Legislature. Accessed at: <https://apps.leg.wa.gov/wac/default.aspx?cite=388-106-0355>. Updated 2019 Oct [accessed 2023 Aug 31] [Context Link 1]
32. TennCare Long Term Services and Supports: A Guide to Pre Admission Evaluation Applications. [Internet] Bureau of TennCare. 2020 Accessed at: <https://www.tn.gov/content/dam/tn/tenncare/documents/PAEManual.pdf>. [accessed 2023 Aug 14] [Context Link 1]
33. Matching multidimensional severity and level of function with type and intensity of service. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. *ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions*. 3rd ed. Carson City, NV: The Change Companies; 2013:69-104. [Context Link 1]
34. Katz S. Assessing self-maintenance: activities of daily living, mobility, and instrumental activities of daily living. *Journal of the American Geriatrics Society* 1983;31(12):721-7. [Context Link 1] View abstract...
35. Yeager JJ. Sleep and activity. In: Meiner SE, Yeager JJ, editors. *Gerontologic Nursing*. 6th ed. St. Louis, MO: Elsevier; 2019:159-173. [Context Link 1]
36. Scott JM. Older adults. In: Rector C, Stanley MJ, editors. *Community and Public Health Nursing Promoting the Public's Health*. 10th ed. Wolters Kluwer; 2022:637-666. [Context Link 1]
37. Fallon T Jr, et al. A level of care instrument for children's systems of care: construction, reliability and validity. *Journal of Child and Family Studies* 2006;15(2):140-152. DOI: 10.1007/s10826-005-9012-y. [Context Link 1]

38. Ouslander JG, et al. Hospital transfers of skilled nursing facility (SNF) patients within 48 hours and 30 days after SNF admission. *Journal of the American Medical Directors Association* 2016;17(9):839-845. DOI: 10.1016/j.jamda.2016.05.021. [Context Link 1] View abstract...
 39. King BJ, Gilmore-Bykovskyi AL, Roiland RA, Polnaszek BE, Bowers BJ, Kind AJ. The consequences of poor communication during transitions from hospital to skilled nursing facility: a qualitative study. *Journal of the American Geriatrics Society* 2013;61(7):1095-102. DOI: 10.1111/jgs.12328. [Context Link 1] View abstract...
 40. Thobaben M. Psychiatric home health care skilled nursing services. *Home Health Care Management & Practice* 2013;25(1):32-4. DOI: 10.1177/1084822312461354. [Context Link 1]
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